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State/Territory Name: New Hampshire

State Plan Amendment (SPA) #: 25-0026

This file contains the following documents in the order listed:

- 1) Approval Letter
- 2) Form CMS-179
- 3) Approved SPA Pages

DEPARTMENT OF HEALTH & HUMAN SERVICES

Centers for Medicare & Medicaid Services

601 E. 12th St., Room 355

Kansas City, Missouri 64106



Medicaid and CHIP Operations Group

March 3, 2026

Lori A. Weaver
Commissioner
Department of Health and Human Services
129 Pleasant Street
Concord, NH 03301

Re: New Hampshire State Plan Amendment (SPA) - 25-0026

Dear Commissioner Weaver:

Enclosed is a corrected approval package for your New Hampshire State Plan Amendment (SPA) submitted under transmittal number (TN) 25-0026. This amendment is to increase prescription drug copayments for Medicaid recipients and remove certain limits to Tobacco Cessation Services, which was approved on February 25, 2026, with an effective date of October 20, 2025. The approval package sent to New Hampshire included the following error:

- Incorrect version of Attachment 3.1-A, Page 2-a

The enclosed corrected package contains the Form CMS-179 and the correct SPA pages to be incorporated into the New Hampshire State Plan.

If you have any questions, please contact Joyce Butterworth at (617) 531-7573 or via email at Joyce.Butterworth@cms.hhs.gov.

Sincerely,



Wendy E. Hill Petras, Acting Director
Division of Program Operations

Enclosures

cc: Henry Lipman, State Medicaid Director
Dawn Tierney, Medicaid Business and Policy

DEPARTMENT OF HEALTH & HUMAN SERVICES

Centers for Medicare & Medicaid Services

601 E. 12th St., Room 355

Kansas City, Missouri 64106



Medicaid and CHIP Operations Group

February 25, 2026

Lori A. Weaver
Commissioner
Department of Health and Human Services
129 Pleasant Street
Concord, NH 03301

Re: New Hampshire State Plan Amendment (SPA) - 25-0026

Dear Commissioner Weaver:

The Centers for Medicare & Medicaid Services (CMS) reviewed your Medicaid State Plan Amendment (SPA) submitted under transmittal number (TN) 25-0026. This amendment proposes to increase prescription drug copayments to \$4.00 for Medicaid recipients and removes certain limits to Tobacco Cession Services.

We conducted our review of your submittal according to statutory requirements in Title XIX and Sections 1916 and 1916A of the Social Security Act and under 42 C.F.R. §447.50-57. This letter informs you that New Hampshire's Medicaid SPA TN 25-0026 was approved on February 25, 2026, with an effective date of October 20, 2025.

Enclosed are copies of Form CMS-179 and approved SPA pages to be incorporated into the New Hampshire State Plan.

If you have any questions, please contact Joyce Butterworth at 617-531-7573 or via email at Joyce.Butterworth@cms.hhs.gov.

Sincerely,

A black rectangular box redacting the signature of Wendy E. Hill Petras.

Wendy E. Hill Petras, Acting Director
Division of Program Operations

Enclosures

cc: Henry Lipman, State Medicaid Director
Dawn Tierney, Medicaid Business and Policy

TRANSMITTAL AND NOTICE OF APPROVAL OF STATE PLAN MATERIAL FOR: CENTERS FOR MEDICARE & MEDICAID SERVICES		1. TRANSMITTAL NUMBER <u>2</u> <u>6</u> <u>5</u> — <u>0</u> <u>0</u> <u>2</u> <u>6</u>	2. STATE <u>NH</u>
		3. PROGRAM IDENTIFICATION: TITLE OF THE SOCIAL SECURITY ACT ☒ XIX ☐ XXI	
TO: CENTER DIRECTOR CENTERS FOR MEDICAID & CHIP SERVICES DEPARTMENT OF HEALTH AND HUMAN SERVICES		4. PROPOSED EFFECTIVE DATE October 20, 2025	
5. FEDERAL STATUTE/REGULATION CITATION <u>42 CFR Part 447.53</u> <u>Sections 1916 and 1916A of the SSA, and 42 C.F.R. 447.50-57</u>		6. FEDERAL BUDGET IMPACT (Amounts in WHOLE dollars) a. FFY <u>2026</u> \$ <u>(1,010)</u> b. FFY <u>2027</u> \$ <u>(1,010)</u>	
7. PAGE NUMBER OF THE PLAN SECTION OR ATTACHMENT Attachment 3.1-A, pages 2, 2-a, 6-b.2 Attachment 3.1-B, pages 2b, 2-c, 5-c.2 Sections G1-G3 G1 - Cost Sharing, pages 1-2 G2a - Cost Sharing, pages 1-2 G2b - Cost Sharing, page 1 G2c - Cost Sharing, pages 1-2 G3 - Cost Sharing - pages 1-5		8. PAGE NUMBER OF THE SUPERSEDED PLAN SECTION OR ATTACHMENT (If Applicable) Attachment 3.1-A, pages 2 and 2-a (14-016) Attachment 3.1-A, page 6-b.2 (23-0036) Attachment 3.1-B, (23-0036) 3.1-B, page 2b and 2-c (14-016) Attachment 3.1-B, (23-0036) page 5-c.2 G1 - Cost Sharing, pages 1-2 (TN 22-0053) Sections G1-G3 (22-0053) G2a - Cost Sharing, pages 1-2 (TN 22-0053) G2b - Cost Sharing, page 1 (TN 22-0053) G2c - Cost Sharing, pages 1-2 (TN 22-0053) G3 - Cost Sharing - pages 1-5 (TN 22-0053)	
9. SUBJECT OF AMENDMENT Pharmacy Co-payment Increase and Removal of Limitation on Tobacco Cessation Services			
10. GOVERNOR'S REVIEW (Check One) ☐ GOVERNOR'S OFFICE REPORTED NO COMMENT ☐ COMMENTS OF GOVERNOR'S OFFICE ENCLOSED ☐ NO REPLY RECEIVED WITHIN 45 DAYS OF SUBMITTAL ☒ OTHER, AS SPECIFIED:			
11. SIGNATURE OF APPROVING OFFICIAL  Christine Santaniello		15. RETURN TO Sara Morris Division of Medicaid Services - Brown Building 129 Pleasant Street Concord, NH 03301	
13. TITLE Associate Commissioner			
14. DATE SUBMITTED December 30, 2025			
FOR CMS USE ONLY			
16. DATE RECEIVED December 30, 2025		17. DATE APPROVED February 25, 2026	
PLAN APPROVED - ONE COPY ATTACHED			
18. EFFECTIVE DATE OF APPROVED MATERIAL October 20, 2025		1 	
20. TYPED NAME OF APPROVING OFFICIAL Wendy E. Hill Patras		2 Acting Director, Division of Program Operations	
22. REMARKS Governor's comments, if any, will follow. 01/08/2026: The State authorized the following pen & ink changes: Box 5: Add federal citations sections 1916 and 1916A of the SSA, and 42 C.F.R. 447.50-57 01/29/2026: The state authorized the following pen & ink changes: Box 1: correct TN number Box 7: correct pagination Box 8: correct pagination			

State/Territory: New HampshireAMOUNT, DURATION AND SCOPE OF MEDICAL
AND REMEDIAL CARE AND SERVICES PROVIDED TO THE CATEGORICALLY NEEDY

- 4.a Nursing facility services (other than services in an institution for mental diseases) for individuals 21 years of age or older.
- Provided: ☐ No limitations ☒ With limitations*
- 4.b. Early and periodic screening, diagnostic and treatment services for individuals under 21 years of age, and treatment of conditions found.*
- 4.c.(i) Family planning services and supplies for individuals of child-bearing age and for individuals eligible pursuant to Attachment 2.2-A, 13 if this eligibility option is elected by the State.*
- Provided: ☒ No limitation ☐ With limitations
- 4.c.(ii) Family planning-related services provided under the above State Eligibility Option*
- Provided: ☒ No limitation ☐ With limitations
- 4.c.l. Tobacco Cessation Counseling Services for Pregnant Women.*
- Provided: ☒ No limitations ☐ With limitations
5. a. Physicians' services whether furnished in the office, the patient's home, a hospital, a nursing facility or elsewhere.
- Provided: ☐ No limitations ☒ With limitations*
- b. Medical and surgical services furnished by a dentist (in accordance with section 1905(a){5}(B) of the Act).
- Provided: ☐ No limitations ☒ With limitations*
6. Medical care and any other type of remedial care recognized under State law, furnished by licensed practitioners within the scope of their practice as defined by State law.
- a. Podiatrists' services.
- Provided: ☐ No limitations ☒ With limitations*

*Description provided on attachment.

4a. Nursing Facility Services

Payment for nursing facility care is available to both categorically and medically needy recipients in need of such care. Payment is made for a non-private room unless a private room is medically necessary. Determination of need for nursing facility care and authorization of payment for nursing facility care is made by the Bureau of Adult and Aging Services. All nursing facility services must be ordered by and provided under the direction of a physician, pursuant to 42 CFR 440.40(a)(1)(iii). Preadmission screen and resident review (PASRR), as applicable, is required for admission to and continued stay in a Medicaid-certified nursing facility, pursuant to 42 CFR Part 483 Subpart C.

4b. Early and Periodic Screening, Diagnosis, and Treatment

Limited to federal requirements for the categorically needy. Any limits to services provided in Attachment 3.1-A do not apply to individuals under EPSDT as long as medical necessity criteria as determined by the Office of Medicaid Business and Policy have been met.

4c. Family Planning Services

4c. (i) Family planning services include those services described in Section 1905(a)(4)(C).

4c. (ii) Family planning-related services provided under the state eligibility option at Attachment 2.2-A B are those medical diagnosis and treatment services provided in a family planning setting as part of, or as follow-up to, a family planning visit pursuant to Section 1902(a)(10)(G)(XVI). Family planning-related services that are covered in NH include: (1) services to treat adverse reactions to, or medical complications of, family planning procedures services, treatment, or therapies (e.g., treatment of perforated uterus due to intrauterine device insertion, treatment of severe menstrual bleeding caused by Depo-Provera injection); (2) drugs (as well as follow-up visits and re-screens based on CDC guidelines) for the treatment of STD's, except for HIV/AIDS and hepatitis, when the STD is identified or diagnosed during a routine or periodic family planning visit; (3) drugs and other treatment (a well as follow-up visit) for lower genital tract and genital skin infections/disorders, and urinary tract infections, when identified/diagnosed during a routine/periodic family planning visit; and (4) vaccinations to prevent cervical cancer routinely provided pursuant to a family planning service in a family planning setting.

4d. Tobacco Cessation Counseling Services for Pregnant Women

Face-to-Face Tobacco Cessation Counseling Services Provided:

- (a) By or under the supervision of a physician; and
- (b) By any other health care professional who is legally authorized to furnish such services under state law and who is authorized to provide Medicaid coverable services *other* than tobacco cessation services; or
- (c) By any other health care professional legally authorized to provide tobacco cessation services under state law *and* who is specifically *designated* by the Secretary in regulations. (None are designated at this time; this item is reserved for future use.)

Please describe any limitations:
None.

AMOUNT, DURATION AND SCOPE OF MEDICAL CARE AND SERVICES PROVIDED

13 a. b. c. d. Other Diagnostic, Screening, Preventative and Rehabilitative Services

13c. Nicotine Cessation Counseling

General Description

Nicotine cessation counseling services are available to educate and support beneficiaries through the process of discontinuing the use of nicotine products.

Pursuant to 42 C.F.R section 440.130(c), nicotine cessation counseling services are provided as preventative services and must be recommended by a physician or other licensed practitioner within the scope of their practice under state law to reduce health risks related to nicotine use and/or promote the physical and mental wellbeing of the beneficiary.

Service Description

The Medicaid program provides coverage for nicotine cessation counseling services to beneficiaries who use nicotine products or who are being treated for nicotine use, with a goal to discontinue the use of nicotine products to improve beneficiary physical and mental health.

Qualified Providers

Nicotine Cessation Counseling Services may be provided by Medicaid enrolled providers that are trained in nicotine cessation counseling services under the following provider types, within their scope of practice according to state law:

- Licensed Physician;
- Licensed Physician Assistant;
- Licensed Osteopath;
- Certified midwife;
- Licensed Advanced Practice Nurse;
- Licensed Mental Health Practitioners: Clinical Mental Health Counselors, Clinical Social Workers, Marriage and Family Therapists, Pastoral Psychotherapists, School Social Workers;
- Pharmacists; and
- Other professional or paraprofessional healthcare practitioners who have completed training in the provision of nicotine cessation counseling and must provide services under the supervision of a licensed practitioner.

TN No: 25-0026
Supersedes
TN No: 23-0036

Approval Date: 02/25/2026

Effective Date: 10/20/2025

State/Territory: New Hampshire

AMOUNT, DURATION AND SCOPE OF SERVICES PROVIDED MEDICALLY NEEDY
GROUP(s): _____

4.d Tobacco Cessation Counseling Services for Pregnant Women.*

Provided: ☒ No limitations ☐ With limitations*

5.a. Physicians' services whether furnished in the office, the patient's home, a hospital, a nursing facility or elsewhere.

Provided: ☐ No limitations ☒ With limitations*

b. Medical and surgical services furnished by a dentist (in accordance with section 1905(a)(5)(B) of the Act).

Provided: ☐ No limitations ☒ With limitations*

*Description provided on attachment.

AMOUNT, DURATION AND SCOPE OF MEDICAL AND REMEDIAL CARE AND SERVICES PROVIDED

Title XIX-NH
Attachment 3.1-B
Page 2-c

4d. Tobacco Cessation Counseling Services for Pregnant Women

Face-to-Face Tobacco Cessation Counseling Services Provided:

- (a) By or under the supervision of a physician; and
- (b) By any other health care professional who is legally authorized to furnish such services under state law and who is authorized to provide Medicaid coverable services *other* than tobacco cessation services; or
- (c) By any other health care professional legally authorized to provide tobacco cessation services under state law *and* who is specifically *designated* by the Secretary in regulations. (None are designated at this time; this item is reserved for future use.)

Please describe any limitations:

None.

5a. Physician Services

Coverage for physician services is unlimited except for those physician services affiliated with outpatient hospital visits, which are limited (in combination with ARNP, clinical psychologist, and pastoral counselor services - 6d) to 12 per recipient per state fiscal year, except for emergency department (ED) and urgent care visits, which are not limited. Limits may be exceeded if prior authorization is granted by the Department based on medical necessity. Laboratory tests and diagnostic x-rays are not counted against the outpatient physician visit limit.

Except for kidney and tissue transplants, to include corneas, bone grafts, and skin transplants, which are covered without prior authorization, prior authorization is required for the coverage of physician services for organ transplants which include bone marrow, liver, heart, lung, heart-lung, pancreas, and pancreas-kidney. (See Attachment 3.1-E for specific details.) Certain surgical procedures to include bariatric surgery, breast reduction, blepharoplasty, panniculectomy, septoplasty and rhinoplasty also require prior authorization.

Non-covered organ transplantation and procurement services include physician services for the surgery, inpatient hospital services for the surgical admission(s), and organ procurement related to the following types of transplants: any type of organ or tissue transplant not specified above (including hairplasty) or more than two transplants of the same type of organ per recipient per lifetime.

In accordance with federal law, coverage for induced abortions is provided when the physician certifies that the pregnancy was the result of rape or incest or the woman suffers from a physical disorder, physical injury, or physical illness including a life-endangering physical condition caused by, or arising from, the pregnancy itself that would place the woman in danger of death unless an abortion is performed.

Physician laboratory and diagnostic radiology services are subject to the limit described under the Other Laboratory and X-ray service heading.

Services provided by licensed psychiatrists and ophthalmologists are included in the physician limits. Payment for refraction is limited to one (1) per recipient per state fiscal year, whether the provider is an optometrist or ophthalmologist.

5b. Medical and Dental Services

Services provided by a doctor of dental surgery or dental medicine which would otherwise be physician services are treated in the same manner as physicians in accordance with 5a.

TN No: 25-0026
Supersedes
TN No: 14-016

Approval Date 02/25/26 Effective Date: 10/20/25

AMOUNT, DURATION AND SCOPE OF MEDICAL AND REMEDIAL CARE AND SERVICES PROVIDED

13 a. b. c. d. Other Diagnostic, Screening, Preventative and Rehabilitative Services

13c. Nicotine Cessation Counseling

General Description

Nicotine cessation counseling services are available to educate and support beneficiaries through the process of discontinuing the use of nicotine products.

Pursuant to 42 C.F.R section 440.130(c), nicotine cessation counseling services are provided as preventative services and must be recommended by a physician or other licensed practitioner within the scope of their practice under state law to reduce health risks related to nicotine use and/or promote the physical and mental wellbeing of the beneficiary.

Service Description

The Medicaid program provides coverage for nicotine cessation counseling services to beneficiaries who use nicotine products or who are being treated for nicotine use, with a goal to discontinue the use of nicotine products to improve beneficiary physical and mental health.

Qualified Providers

Nicotine Cessation Counseling Services may be provided by Medicaid enrolled providers that are trained in nicotine cessation counseling services under the following provider types, within their scope of practice according to state law:

- Licensed Physician;
- Licensed Physician Assistant;
- Licensed Osteopath;
- Certified midwife;
- Licensed Advanced Practice Nurse;
- Licensed Mental Health Practitioners: Clinical Mental Health Counselors, Clinical Social Workers, Marriage and Family Therapists, Pastoral Psychotherapists, School Social Workers;
- Pharmacists; and
- Other professional or paraprofessional healthcare practitioners who have completed training in the provision of nicotine cessation counseling and must provide services under the supervision of a licensed practitioner.

TN No: 25-0026
Supersedes
TN No: 23-0036

Approval Date: 02/25/2026

Effective Date: 10/20/2025



Medicaid Premiums and Cost Sharing

State Name: New Hampshire

OMB Control Number: 0938-1148

Transmittal Number: NH - 25 - 0026

Cost Sharing Requirements

G1

1916
1916A
42 CFR 447.50 through 447.57 (excluding 447.55)

The state charges cost sharing (deductibles, co-insurance or co-payments) to individuals covered under Medicaid.

Yes

- ☒ The state assures that it administers cost sharing in accordance with sections 1916 and 1916A of the Social Security Act and 42 CFR 447.50 through 447.57.

General Provisions

- ☒ The cost sharing amounts established by the state for services are always less than the amount the agency pays for the service.
- ☐ No provider may deny services to an eligible individual on account of the individual's inability to pay cost sharing, except as elected by the state in accordance with 42 CFR 447.52(e)(1).
- ☐ The process used by the state to inform providers whether cost sharing for a specific item or service may be imposed on a beneficiary and whether the provider may require the beneficiary to pay the cost sharing charge, as a condition for receiving the item or service, is (check all that apply):
- ☒ The state includes an indicator in the Medicaid Management Information System (MMIS)
 - ☒ The state includes an indicator in the Eligibility and Enrollment System
 - ☒ The state includes an indicator in the Eligibility Verification System
 - ☐ The state includes an indicator on the Medicaid card, which the beneficiary presents to the provider
 - ☐ Other process
- ☐ Contracts with managed care organizations (MCOs) provide that any cost-sharing charges the MCO imposes on Medicaid enrollees are in accordance with the cost sharing specified in the state plan and the requirements set forth in 42 CFR 447.50 through 447.57.

Cost Sharing for Non-Emergency Services Provided in a Hospital Emergency Department

The state imposes cost sharing for non-emergency services provided in a hospital emergency department.

No

Cost Sharing for Drugs

The state charges cost sharing for drugs.

Yes

The state has established differential cost sharing for preferred and non-preferred drugs.

No

- ☐ All drugs will be considered preferred drugs.



Medicaid Premiums and Cost Sharing

Beneficiary and Public Notice Requirements

- ☒ Consistent with 42 CFR 447.57, the state makes available a public schedule describing current cost sharing requirements in a manner that ensures that affected applicants, beneficiaries and providers are likely to have access to the notice. Prior to submitting a SPA which establishes or substantially modifies existing cost sharing amounts or policies, the state provides the public with advance notice of the SPA, specifying the amount of cost sharing and who is subject to the charges, and provides reasonable opportunity for stakeholder comment. Documentation demonstrating that the notice requirements have been met are submitted with the SPA. The state also provides opportunity for additional public notice if cost sharing is substantially modified during the SPA approval process.

Other Relevant Information

PRA Disclosure Statement

According to the Paperwork Reduction Act of 1995, no persons are required to respond to a collection of information unless it displays a valid OMB control number. The valid OMB control number for this information collection is 0938-1148. The time required to complete this information collection is estimated to average 40 hours per response, including the time to review instructions, search existing data resources, gather the data needed, and complete and review the information collection. If you have comments concerning the accuracy of the time estimate(s) or suggestions for improving this form, please write to: CMS, 7500 Security Boulevard, Attn: PRA Reports Clearance Officer, Mail Stop C4-26-05, Baltimore, Maryland 21244-1850.

V.20160722



Medicaid Premiums and Cost Sharing

State Name: New Hampshire

OMB Control Number: 0938-1148

Transmittal Number: NH - 25 - 0026

Cost Sharing Amounts - Categorically Needy Individuals

G2a

1916
1916A
42 CFR 447.52 through 54

The state charges cost sharing to all categorically needy (Mandatory Coverage and Options for Coverage) individuals.

Yes

Services or Items with the Same Cost Sharing Amount for All Incomes

Add	Service or Item	Amount	Dollars or Percentage	Unit	Explanation	Remove
Add						Remove

Services or Items with Cost Sharing Amounts that Vary by Income

Service or Item: Adult Dental Services

Remove Service or Item

Indicate the income ranges by which the cost sharing amount for this service or item varies.

Add	Incomes Greater than	Incomes Less than or Equal to	Amount	Dollars or Percentage	Unit	Explanation	Remove
Add	100% FPL		10.00	%	Visit	restorative, oral surgery, and limited periodontic services	Remove

Service or Item: Prescription Drugs

Remove Service or Item

Indicate the income ranges by which the cost sharing amount for this service or item varies.

Add	Incomes Greater than	Incomes Less than or Equal to	Amount	Dollars or Percentage	Unit	Explanation	Remove
Add	100% FPL		4.00	\$	Prescription		Remove

Add Service or Item

Cost Sharing for Non-preferred Drugs Charged to Otherwise Exempt Individuals

If the state charges cost sharing for non-preferred drugs (entered above), answer the following question:

The state charges cost sharing for non-preferred drugs to otherwise exempt individuals.

No

Cost Sharing for Non-emergency Services Provided in the Hospital Emergency Department Charged to Otherwise Exempt Individuals

If the state charges cost sharing for non-emergency services provided in the hospital emergency department (entered above), answer the following question:



Medicaid Premiums and Cost Sharing

The state charges cost sharing for non-emergency services provided in the hospital emergency department to otherwise exempt individuals.

No

PRA Disclosure Statement

According to the Paperwork Reduction Act of 1995, no persons are required to respond to a collection of information unless it displays a valid OMB control number. The valid OMB control number for this information collection is 0938-1148. The time required to complete this information collection is estimated to average 40 hours per response, including the time to review instructions, search existing data resources, gather the data needed, and complete and review the information collection. If you have comments concerning the accuracy of the time estimate(s) or suggestions for improving this form, please write to: CMS, 7500 Security Boulevard, Attn: PRA Reports Clearance Officer, Mail Stop C4-26-05, Baltimore, Maryland 21244-1850.

V.20181119



Medicaid Premiums and Cost Sharing

State Name:

OMB Control Number: 0938-1148

Transmittal Number: NH - 25 - 0026

Cost Sharing Amounts - Medically Needy Individuals

G2b

1916
1916A
42 CFR 447.52 through 54

The state charges cost sharing to all medically needy individuals.

No

PRA Disclosure Statement

According to the Paperwork Reduction Act of 1995, no persons are required to respond to a collection of information unless it displays a valid OMB control number. The valid OMB control number for this information collection is 0938-1148. The time required to complete this information collection is estimated to average 40 hours per response, including the time to review instructions, search existing data resources, gather the data needed, and complete and review the information collection. If you have comments concerning the accuracy of the time estimate(s) or suggestions for improving this form, please write to: CMS, 7500 Security Boulevard, Attn: PRA Reports Clearance Officer, Mail Stop C4-26-05, Baltimore, Maryland 21244-1850.

V.20181119



Medicaid Premiums and Cost Sharing

State Name: New Hampshire

OMB Control Number: 0938-1148

Transmittal Number: NH - 25 - 0026

Cost Sharing Amounts - Targeting

G2c

1916
1916A
42 CFR 447.52 through 54

The state targets cost sharing to a specific group or groups of individuals.

Yes

Population Name (optional):

Eligibility Group(s) Included: all adult eligibility groups not carved out via 42 CFR 447.56 excluded groups

Incomes Greater than 100% TO Incomes Less than or Equal to

Add	Service	Amount	Dollars or Percentage	Unit	Explanation	Remove
Add	Adult Dental Services	10.00	%	Visit	restorative, oral surgery, and limited periodontic services	Remove

The state permits providers to require individuals to pay cost sharing as a condition for receiving items or services, subject to the conditions specified at 42 CFR 447.52(e)(1). This is only permitted for non-exempt individuals with family income above 100% FPL.

Yes

Providers may require payment of cost sharing as a condition for receiving all items or services listed above.

Yes

Cost Sharing for Non-preferred Drugs Charged to Otherwise Exempt Individuals

If the state targets cost sharing for non-preferred drugs to specific groups of individuals (entered above), answer the following question:

The state charges cost sharing for non-preferred drugs to otherwise exempt individuals.

Cost Sharing for Non-emergency Services Provided in the Hospital Emergency Department Charged to Otherwise Exempt Individuals

If the state charges cost sharing for non-emergency services provided in the hospital emergency department to specific individuals (entered above), answer the following question:

The state charges cost sharing for non-emergency services provided in the hospital emergency department to otherwise exempt individuals.

Remove Population

Add Population



Medicaid Premiums and Cost Sharing

PRA Disclosure Statement

According to the Paperwork Reduction Act of 1995, no persons are required to respond to a collection of information unless it displays a valid OMB control number. The valid OMB control number for this information collection is 0938-1148. The time required to complete this information collection is estimated to average 40 hours per response, including the time to review instructions, search existing data resources, gather the data needed, and complete and review the information collection. If you have comments concerning the accuracy of the time estimate(s) or suggestions for improving this form, please write to: CMS, 7500 Security Boulevard, Attn: PRA Reports Clearance Officer, Mail Stop C4-26-05, Baltimore, Maryland 21244-1850.

V.20181119



Medicaid Premiums and Cost Sharing

State Name: New Hampshire

OMB Control Number: 0938-1148

Transmittal Number: NH - 25 - 0026

Cost Sharing Limitations

G3

42 CFR 447.56
1916
1916A

- ☒ The state administers cost sharing in accordance with the limitations described at 42 CFR 447.56, and 1916(a)(2) and (j) and 1916A(b) of the Social Security Act, as follows:

Exemptions

Groups of Individuals - Mandatory Exemptions

The state may not impose cost sharing upon the following groups of individuals:

- ☒ Individuals ages 1 and older, and under age 18 eligible under the Infants and Children under Age 18 eligibility group (42 CFR 435.118).
- ☒ Infants under age 1 eligible under the Infants and Children under Age 18 eligibility group (42 CFR 435.118), whose income does not exceed the higher of:
 - ☒ 133% FPL; and
 - ☒ If applicable, the percent FPL described in section 1902(l)(2)(A)(iv) of the Act, up to 185 percent.
- ☒ Disabled or blind individuals under age 18 eligible for the following eligibility groups:
 - ☒ SSI Beneficiaries (42 CFR 435.120).
 - ☒ Blind and Disabled Individuals in 209(b) States (42 CFR 435.121).
 - ☒ Individuals Receiving Mandatory State Supplements (42 CFR 435.130).
- ☒ Children for whom child welfare services are made available under Part B of title IV of the Act on the basis of being a child in foster care and individuals receiving benefits under Part E of that title, without regard to age.
- ☒ Disabled children eligible for Medicaid under the Family Opportunity Act (1902(a)(10)(A)(ii)(XIX) and 1902(cc) of the Act).
- ☒ Pregnant women, during pregnancy and through the postpartum period which begins on the last day of pregnancy and extends through the end of the month in which the 60-day period following termination of pregnancy ends, except for cost sharing for services specified in the state plan as not pregnancy-related.
- ☒ Any individual whose medical assistance for services furnished in an institution is reduced by amounts reflecting available income other than required for personal needs.
- ☒ An individual receiving hospice care, as defined in section 1905(o) of the Act.
- ☒ Indians who are currently receiving or have ever received an item or service furnished by an Indian health care provider or through referral under contract health services.
- ☒ Individuals who are receiving Medicaid because of the state's election to extend coverage to the Certain Individuals Needing Treatment for Breast or Cervical Cancer eligibility group (42 CFR 435.213).



Medicaid Premiums and Cost Sharing

Groups of Individuals - Optional Exemptions

The state may elect to exempt the following groups of individuals from cost sharing:

The state elects to exempt individuals under age 19, 20 or 21, or any reasonable category of individuals 18 years of age or over.

No

The state elects to exempt individuals whose medical assistance for services furnished in a home and community-based setting is reduced by amounts reflecting available income other than required for personal needs.

Yes

Services - Mandatory Exemptions

The state may not impose cost sharing for the following services:

- ☒ Emergency services as defined at section 1932(b)(2) of the Act and 42 CFR 438.114(a).
- ☒ Family planning services and supplies described in section 1905(a)(4)(C) of the Act, including contraceptives and pharmaceuticals for which the state claims or could claim federal match at the enhanced rate under section 1903(a)(5) of the Act for family planning services and supplies.
- ☒ Preventive services, at a minimum the services specified at 42 CFR 457.520, provided to children under 18 years of age regardless of family income, which reflect the well-baby and well child care and immunizations in the Bright Futures guidelines issued by the American Academy of Pediatrics.
- ☒ Pregnancy-related services, including those defined at 42 CFR 440.210(a)(2) and 440.250(p), and counseling and drugs for cessation of tobacco use. All services provided to pregnant women will be considered pregnancy-related, except those services specifically identified in the state plan as not being related to pregnancy.
- ☒ Provider-preventable services as defined in 42 CFR 447.26(b).

Enforceability of Exemptions

The procedures for implementing and enforcing the exemptions from cost sharing contained in 42 CFR 447.56 are (check all that apply):

- ☒ To identify that American Indians/Alaskan Natives (AI/AN) are currently receiving or have ever received an item or service furnished by an Indian health care provider or through referral under contract health services in accordance with 42 CFR 447.56(a)(1)(x), the state uses the following procedures:
 - ☒ The state accepts self-attestation
 - ☐ The state runs periodic claims reviews
 - ☐ The state obtains an Active or Previous User Letter or other Indian Health Services (IHS) document
 - ☐ The Eligibility and Enrollment and MMIS systems flag exempt recipients
 - ☐ Other procedure

Additional description of procedures used is provided below (optional):

The state will rely on the following question in the single streamlined application: "Has this person ever gotten a service from the Indian Health Service, a tribal health program, or urban Indian Health Program, or through a referral from one of these programs?" Any individual who answers "yes" will be exempt from cost-sharing.

- ☒ To identify all other individuals exempt from cost sharing, the state uses the following procedures (check all that apply):



Medicaid Premiums and Cost Sharing

- ☒ The MMIS system flags recipients who are exempt
- ☐ The Eligibility and Enrollment System flags recipients who are exempt
- ☐ The Medicaid card indicates if beneficiary is exempt
- ☒ The Eligibility Verification System notifies providers when a beneficiary is exempt
- ☐ Other procedure

Additional description of procedures used is provided below (optional):

Members that have cost sharing are flagged with an indicator which includes the date span to which the cost sharing applies. It is possible for a member to have multiple cost sharing spans with different dates due to the tracking of cost sharing, which occurs quarterly. This indicator and date spans are sent from the New HEIGHTS eligibility system to the MMIS. The MMIS stores this information and sends it to our managed care organizations, the dental prepaid ambulatory health plan (PAHP), and our PBM for use in claims payment. Through EVS (electronic verification), providers are able to identify if the member has a cost sharing responsibility.

Payments to Providers

- ☒ The state reduces the payment it makes to a provider by the amount of a beneficiary's cost sharing obligation, regardless of whether the provider has collected the payment or waived the cost sharing, except as provided under 42 CFR 447.56(c).

Payments to Managed Care Organizations

The state contracts with one or more managed care organizations to deliver services under Medicaid.

Yes

- ☒ The state calculates its payments to managed care organizations to include cost sharing established under the state plan for beneficiaries not exempt from cost sharing, regardless of whether the organization imposes the cost sharing on its recipient members or the cost sharing is collected.

Aggregate Limits

- ☒ Medicaid premiums and cost sharing incurred by all individuals in the Medicaid household do not exceed an aggregate limit of 5 percent of the family's income applied on a quarterly or monthly basis.
 - ☐ The percentage of family income used for the aggregate limit is:
 - ☒ 5%
 - ☐ 4%
 - ☐ 3%
 - ☐ 2%
 - ☐ 1%
 - ☐ Other: %
 - ☐ The state calculates family income for the purpose of the aggregate limit on the following basis:
 - ☒ Quarterly



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☐ Monthly

The state has a process to track each family's incurred premiums and cost sharing through a mechanism that does not rely on beneficiary documentation.

Yes

☒ Describe the mechanism by which the state tracks each family's incurred premiums and cost sharing (check all that apply):

☒ As claims are submitted for dates of services within the family's current monthly or quarterly cap period, the state applies the incurred cost sharing for that service to the family's aggregate limit. Once the family reaches the aggregate limit, based on incurred cost sharing and any applicable premiums, the state notifies the family and providers that the family has reached their aggregate limit for the current monthly or quarterly cap period, and are no longer subject to premiums or cost sharing.

☒ Managed care organization(s) track each family's incurred cost sharing, as follows:

Managed care organizations and/or the PAHP will provide data to state systems that will allow the Medicaid agency to track the incurred cost-sharing across all services for beneficiaries who are copay eligible.

☐ Other process:

☒ Describe how the state informs beneficiaries and providers of the beneficiaries' aggregate family limit and notifies beneficiaries and providers when a beneficiary has incurred premiums and cost sharing up to the aggregate family limit and individual family members are no longer subject to premiums or cost sharing for the remainder of the family's current monthly or quarterly cap period:

Beneficiaries who are identified by the Medicaid agency as having met their quarterly cap will be updated in the state eligibility system as no longer copay eligible for the remainder of the quarter and this update will be reflected in the eligibility files sent to managed care organizations and/or the PAHP. This update will also generate a letter to the beneficiary to notify them that they are no longer copay eligible and specify when their copay eligibility will restart. Providers who may collect copays under the Medicaid program will be able to use provider-facing portals to identify a beneficiary as copay eligible or not.

The state has a documented appeals process for families that believe they have incurred premiums or cost sharing over the aggregate limit for the current monthly or quarterly cap period.

Yes

Describe the appeals process used:

Beneficiaries may submit information to indicate they have incurred cost sharing over the aggregate limit. This will be validated against state systems and beneficiary will be reimbursed if the limit is exceeded.

☒ Describe the process used to reimburse beneficiaries and/or providers if the family is identified as paying over the aggregate limit for the month/quarter:

Beneficiaries will be made aware in cost-sharing notifications that they may notify the Medicaid agency if they have identified themselves as paying over the aggregate limit. The Medicaid agency will also track aggregate cost-sharing to identify over-payments and reimburse those over-payments.

☒ Describe the process for beneficiaries to request a reassessment of their family aggregate limit if they have a change in circumstances or if they are being terminated for failure to pay a premium:



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At any time, enrollees may notify the Medicaid agency of a change in income or other circumstances that might change, their aggregate cost-sharing limit. Once a beneficiary notifies the Medicaid agency of such a change, the Medicaid agency will review the updated information and change the aggregate limits, if necessary.

The state imposes additional aggregate limits, consistent with 42 CFR 447.56(f)(5).

No

PRA Disclosure Statement

According to the Paperwork Reduction Act of 1995, no persons are required to respond to a collection of information unless it displays a valid OMB control number. The valid OMB control number for this information collection is 0938-1148. The time required to complete this information collection is estimated to average 40 hours per response, including the time to review instructions, search existing data resources, gather the data needed, and complete and review the information collection. If you have comments concerning the accuracy of the time estimate(s) or suggestions for improving this form, please write to: CMS, 7500 Security Boulevard, Attn: PRA Reports Clearance Officer, Mail Stop C4-26-05, Baltimore, Maryland 21244-1850.

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